Carboplatin + fluorouracil after nasopharyngeal chemoradiotherapy

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Adjuvant carboplatin / fluorouracil

Active medicines: Carboplatin + Fluorouracil

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2 and 3	Once daily on the stated days NCIS reference; review interactions and the actual multi-day infusion plan.
Ondansetron	16 mg orally Day 1	Before chemotherapy Clinician may split to 8 mg twice daily or reduce to 8 mg once daily. Review QT risk and electrolytes.
Dexamethasone · reconcile total steroid prescription	NCIS reference: 12 mg orally Day 1; 8 mg orally Days 2–4	As prescribed Individualise for diabetes, interactions, infection risk and sleep effects.
Metoclopramide · duration to be specified	10 mg orally three times daily in the NCIS subsequent-days row	Exact duration and indication must be prescribed The high-risk row does not state an end day or PRN qualifier. Check renal/neurological suitability; avoid unreviewed stacking with other dopamine antagonists.

Additional phase-specific support

- Carboplatin dosing uses the selected AUC and a verified absolute kidney-function estimate in mL/min. Indexed laboratory estimated glomerular filtration rate (kidney filtration) (eGFR) must not be substituted directly into Calvert dosing.
- Check the kidney-function method, creatinine timing, prior platinum exposure and any previous hypersensitivity. Severe low counts and renal impairment may change the next treatment plan.
- Record the dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) result before the first fluorouracil exposure unless previously tested. Complete deficiency precludes fluorouracil; partial deficiency needs an individual starting dose and careful subsequent review.
- Normal DPD testing cannot exclude all severe fluorouracil toxicity. Explain unexpectedly early or severe diarrhoea, mucositis, neurological symptoms and chest pain.
- Write the actual infusion option, access arrangement, bag or pump changes and disconnect time. Do not equate four treatment days with a single universal pump duration.
- Provide an individually prescribed diarrhoea plan and hand/foot care advice; severe or persistent symptoms require assessment rather than repeated self-treatment.
- Arrange nutrition, weight and swallowing review early. Dietitian and speech and language therapy input should follow the person's needs; swallowing safety cannot be inferred from a general leaflet.
- Use the agreed mouth-care and dental plan. Report painful swallowing, mouth ulceration, weight loss or inability to drink before the next appointment.
- Reconcile prescription, over-the-counter and complementary medicines. Discuss fertility, contraception and breastfeeding using the actual products and individual circumstances.
- Confirm the hospital's urgent contact and fever instructions. Infection, bleeding, severe diarrhoea, dehydration, chest pain or new breathlessness need prompt assessment, not the next routine review.
- NCIS uses the dedicated carboplatin AUC≥4 antiemetic row. Routine olanzapine is not included in that row. Cover the remaining fluorouracil infusion days according to daily emetic risk and individual response.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00552 v6 · pNCCP 00552: treatment and supportive-care sections; NCIS v8 pp4–5 where used · NCCP 00552 and current NCIS/local supportive-care guidance

Sources checked 2026-11-10 · SACTCheck v0.84.47 · Regimen-level drafts; prescriber and oncology-pharmacy verification required